PUBLIC EXECUTIVE SUMMARY · 03 IN READING ORDER

NHS

/ PRIMARY CARE

REDESIGN EXECUTIVE SUMMARY

Continuity replacement logic — the corrected primitive, minimum replacement and proof standard.

> *"The NHS has records. What it has not ensured is that they arrive where clinical decisions are made."*

ARTIFACT FAMILY

#	ARTIFACT	ACCESS
01	Translation Artifact	Public
02	Diagnostic Artifact	Public
03	**Redesign Executive Summary**	← *This document*
04	Full Redesign Artifact	Restricted

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CLASSIFICATION Public · 03 in reading order · not for implementation

SECTION 0

WHAT THIS DOCUMENT IS

Primary care is record-rich and continuity-poor. Patients repeat their history. GPs reconstruct instead of judging. The same harm pattern recurs. This document presents a structural replacement that corrects the primitive: operational continuity must be present at the point of clinical consequence, in patient-held form.

This is the public executive summary of a structural replacement specification for the NHS primary care continuity failure. It is not a reform proposal, a digital access programme, or an optimisation of the existing system. It sets out the minimum architecture the Full Redesign Artifact specifies to stop the failure reproducing, and the proof standard that would show whether it has.

> **ARTIFACT FAMILY — READING ORDER**
>
> Translation Artifact (start here) → Diagnostic Artifact (full evidence) → this document (replacement logic) → Full Redesign Artifact (restricted; not published). Public artifacts demonstrate the diagnostic and replacement logic. The restricted artifact specifies the implementation.

What this document contains: corrected primitive, truth kernel, target-state architecture, minimum valid replacement, what must stop, dependency order, fake progress test, consequence of deferral, proof conditions, board verdict.

What the Full Redesign Artifact adds: the complete seven-move transition sequence, rules and invariants, drift gates that fail closed, admissibility conditions, successor-trap analysis, builder handoff with object-level specifications, and testability requirements. The Full Redesign Artifact is restricted and not publicly available. It is a governing specification — ready to seed a build backlog, not a build-ready engineering specification. Wireframes, API contracts, integration designs and the formal assurance cases (clinical safety, data protection, security) are separate work.

Judgment boundary

This document makes no claim about clinical negligence, individual clinician failure, institutional bad faith or adjudicated liability. It is a structural specification derived from public evidence. The failures addressed are architectural, not personal.

What this document does not say

- → It does not abolish the 10-minute consultation — it changes what arrives before it
- → It does not require same-GP continuity — it requires operational memory to be present whichever clinician sees the patient
- → It does not create a diagnostic tool, medical device or clinical authority
- → It does not replace the official NHS GP record — the app is a patient-held continuity layer; formal ingestion is Move 7
- → It does not require NHS IT cooperation for the redesign to deliver core continuity value
- → It does not prescribe a technology vendor or EHR solution
- → It does not claim patient-facing records are new — platforms such as Patients Know Best and the NHS App already exist; Section 3 sets out what this redesign adds
- → It does not claim access to unpublished NHS internal data

SECTION 0.1

REDESIGN SPINE

The replacement specification in governed form.

REDESIGN ELEMENT	SPECIFICATION
Governing mismatch	Record-rich and continuity-poor: audit trace is extensive; operational memory is not reliably present at the point of clinical consequence
What must become true	Five diagnostic conditions: continuity named as a requirement; operational record separated from audit; usable history before the decision; outcomes and handovers carried forward; continuity that holds when the patient cannot carry it
Corrected primitive	Operational continuity must be present at the point of clinical consequence, in patient-held form
Truth kernel	The patient's portable operational continuity record — the one object the redesign must not sacrifice
Continuity layer	The patient holds the record; the app prepares a consultation packet before each encounter; the outcome is appended afterwards
Clinical authority layer	The GP receives the packet, exercises clinical judgment and keeps full medical authority
Core mechanism	Consultation packet: a one-page summary readable in under 60 seconds, so the appointment begins at clinical judgment, not reconstruction
Non-negotiable (within this route)	Patient-held custody. The app carries continuity, not clinical authority. If custody transfers to an institution, this has become a different architecture and must meet the institutional conditions in Section 3.
Minimum replacement	Continuity record + symptom event layer (capture; routing prompts optional until classified) + consultation packet + outcome append + emergency access, all present together; carer access and non-digital fallback apply throughout. See Section 5.
Proof condition	Same pressure → different response. Against baseline, patients no longer repeat their history by default; the GP begins at clinical judgment.
Primary recapture risk	The app becomes a health journal under engagement-metric pressure; or an institution takes custody and recreates fragmentation

SECTION 1

GOVERNING DIAGNOSIS

The NHS has records. What it has not ensured is that they arrive where clinical decisions are made. Primary care is record-rich and continuity-poor. Patients repeat their history. GPs reconstruct instead of judging. The same harm pattern recurs.

GOVERNING DIAGNOSIS — REDESIGN BASIS

NHS primary care is record-rich and continuity-poor: it has built extensive audit trace without ensuring that operational memory is retrievable at the point of clinical consequence. Relational continuity — seeing the same GP — was the human patch for this architectural absence. As that kind of continuity has declined, the patch has not held. The memory function has to be built into the architecture.

The patient becomes the record. When the system does not retrieve its own memory, the patient supplies history, the carer supplies context, and the GP reconstructs the case. The system supplies fragments. This is not a clinician failure. It is a structural mismatch between what the NHS promises and what its architecture reliably delivers.

The architecture is event-based — optimised for the appointment, the referral, the discharge. The patient is state-based — a continuous clinical condition that exists between appointments as much as during them. Continuity fails when a state-bearing person is managed through an event-based ledger and nothing carries the state.

The reforms reviewed in the Diagnostic improved documentation, access or sharing without establishing that usable continuity reaches the decision. The proposed Single Patient Record faces the same test: whether it is measured by data assembled, or by usable continuity at the point of decision.

What must become true

The Diagnostic Artifact sets out five conditions, in dependency order. They do not depend on any particular technology. The redesign summarised here is one route to them.

DIAGNOSTIC CONDITION	HOW THIS REDESIGN MEETS IT
1 — Continuity named as a requirement	The corrected primitive and the truth kernel define the continuity object and its minimum standard (Move 1)
2 — Operational record separated from audit	An append-only continuity record built for retrieval, not compliance (Moves 1 and 1b)
3 — Usable history before the decision	Symptom event layer and consultation packet, including repeat presentations (Moves 2 and 3)
4 — Outcomes and handovers carried forward	Outcome append after each encounter; institutional data such as discharge summaries once recognised (Moves 4 and 7)
5 — Holds when the patient cannot carry it	Emergency access, carer access and non-digital fallback (Moves 5 and 6)

SECTION 2

TRUTH KERNEL

The continuity object this redesign must preserve is the patient’s portable operational continuity record. It is not a feature of the app. It is what the app exists to hold.

The record follows the person, not the institution. It carries clinical history, symptom events, clinical outcomes, medications, referrals and follow-ups across encounters, transitions and care settings. No organisation owns it exclusively. No transition should destroy it.

Continuity builds up over time. Each appended outcome makes the next consultation better prepared. A patient attending their fifth appointment in six months arrives with a symptom timeline, prior clinical outcomes and a structured view of the current episode — not because they remembered it, but because the system held it.

> **PATIENT-HELD ≠ PATIENT-BURDENED**
>
> ***Patient-held does not mean patient-burdened. The patient holds custody of the continuity record; the architecture must reduce the labour required to keep, retrieve and present it. Any design that increases patient administrative burden while calling it empowerment fails the redesign.***

Kernel test: if the redesign is complete but the continuity record is still fragmented across institutional systems, is not retrievable in under 60 seconds at the point of care, or custody sits with an institution rather than the patient, the redesign has not succeeded, whatever else has improved.

SECTION 3

CORRECTED PRIMITIVE

	PRIMITIVE
Malformed primitive	The NHS provides continuity of care through electronic records held by the institutions that generated them — with the patient carrying whatever does not arrive.
Corrected primitive	Operational continuity must be present at the point of clinical consequence, in patient-held form, readable by any clinician at any encounter, independent of institutional interoperability.

The correction is not to make the institutional record architecture clearer, faster or better connected. Those improvements remain useful. The correction is to change who holds the continuity object and where it arrives.

Why patient-held

The Diagnostic's five conditions do not depend on any technology. Patient-held custody is a design choice for this route, and it rests on four arguments:

- → **The patient is the only thing present at every encounter** — every practice, hospital, out-of-hours service and emergency. A record that travels with the person crosses every organisational boundary by default.
- → **It can start now.** It does not wait for interoperability, procurement or institutional permission; NHS integration is a later step (Move 7), not a prerequisite.
- → **It formalises a role the patient already carries** — and is required to make that role lighter, not heavier (patient-held ≠ patient-burdened).
- → **It resists recapture.** Organisation-held custody is how the current fragmentation arose; patient custody keeps the continuity object outside any single institution's boundary.

The costs are real and are designed for in the Full Redesign: digital exclusion (carer access and non-digital fallback), coercion (sensitive mode and revocable access), the quality of patient-entered information (provenance tags, clinician verification, dispute flags) and clinician trust (the 60-second packet and a clinician-adoption boundary).

When an institutional route would also satisfy the diagnosis

An institutional route — such as the proposed Single Patient Record — could meet the five conditions if it: is measured by usable continuity at the point of decision, not data assembled; presents a decision-ready summary readable in under 60 seconds to any clinician, including in emergency and out-of-hours settings; carries outcomes and handovers across organisational boundaries without request-based joins; works when the patient cannot speak; and does not move retrieval work back onto the patient. The two routes are not exclusive: through the One-Way Valve (Section 4), institutional records can feed the patient-held layer.

Relation to existing products

Patient-facing record platforms already exist. Patients Know Best, for example, is live in about half of England's regions with over 8 million registered patients and supplies hospital test results to the NHS App; the NHS App itself gives patients access to their GP record. This redesign does not claim that patient-held records are new. What it specifies is narrower and stricter: a record organised for the next clinical decision (the 60-second consultation packet), outcomes appended after each encounter, symptom capture before it, and refusal invariants on custody, engagement metrics and diagnostic creep. In a live engagement, whether to build, buy or extend an existing platform is a decision tested against those requirements.

SECTION 4

TARGET-STATE ARCHITECTURE

The redesigned system is a dual-layer continuity architecture.

LAYER	WHAT CHANGES	THE APP'S ROLE
Continuity layer (patient-held)	The patient holds a portable operational continuity record. The app prepares a consultation packet before each encounter and receives the outcome afterwards. The record travels across care settings.	Holds the continuity object. Moves sensing upstream. Prepares the consultation packet. Receives outcome appends.
Clinical authority layer (GP consultation)	The GP receives the packet before or at the start of the encounter, exercises clinical judgment on structured prior state, and the outcome is appended after the encounter.	Carries continuity, not clinical authority. It does not diagnose, prescribe or refer. It prepares the encounter.

The consultation packet — the core mechanism

The consultation packet is the mechanism that addresses the first structural break directly. It is a one-page summary prepared by the app from the patient's continuity record and presented to the clinician at the start of an encounter.

Clinical signal filter. The packet is not a data dump. It surfaces the highest-signal recent changes — the last clinical outcome, outstanding investigations, medications changed since the last visit, unresolved presentations, and the current symptom episode. A clinician should never need to read the full record to be ready to act.

What the packet contains (high level): current symptom event and severity; last clinical outcome and date; active medications and recent changes; outstanding investigations and pending follow-ups; known allergies and red-flag conditions; the patient's own adherence notes. Every entry shows who entered it — patient, carer or clinician.

CURRENT ENCOUNTER	REDESIGNED ENCOUNTER
GP opens the encounter cold; the patient repeats their history; reconstruction uses up the appointment	GP reads the packet in under 60 seconds; the appointment begins at clinical judgment
Clinical judgment squeezed into whatever time reconstruction leaves	More of the appointment available for clinical judgment on structured prior state
Outcome not reliably recorded or available to the next clinician	Outcome appended to the continuity record; it appears in the next packet

> **60-SECOND RETRIEVAL RULE**
>
> If the packet takes longer than 60 seconds for a clinician to read and act on, the redesign has not delivered the mechanism. This is a hard constraint, not a guideline.

Admissibility boundary

At launch, the patient's continuity record is not the official NHS GP record. It is a patient-held operational memory object; the official GP record remains the medico-legal record. Formal NHS recognition is a later migration step — Move 7 in the Full Redesign Artifact — not an assumption. The app must be useful before formal integration, and must not need NHS permission to deliver core continuity value.

One-Way Valve. When formal integration begins, NHS systems may push data to the continuity record (for example SCR updates and discharge summaries). No NHS system may delete, overwrite or take custody of the patient-held record. The portable record is patient-controlled. Institutions contribute to it; they do not own it.

SECTION 5

MINIMUM VALID REPLACEMENT

The redesign is only real if all five of the following exist together. A deployment missing any one of them is not a replacement. It is a partial correction inside the old frame.

1. **Patient-held operational continuity record** — portable, persistent, retrievable in under 60 seconds at any point of care
2. **Symptom event layer** — structured symptom capture before the encounter; creates the information the consultation packet draws on. Routing prompts are optional and released only once their regulatory classification is settled
3. **Consultation packet** — one-page summary prepared before the encounter; a clinician reads it in under 60 seconds without verbal reconstruction
4. **Outcome append** — post-encounter record of clinical outcome, prescriptions, referrals and follow-up, preserved for the next encounter
5. **Emergency access** — essential fields available to first responders without login, including through a printed Continuity Card

Inclusion requirements across all five. Carer access, with explicit role tagging, and non-digital fallback — PDF export, a printed packet at the practice, and the Continuity Card — so that digitally excluded, elderly and cognitively impaired patients receive the same benefit. They are part of the minimum replacement, not extras.

MINIMUM REPLACEMENT RULE

If any one of these five components is missing, the change is not yet a redesign. It is a partial correction inside the old frame — and the old failure remains liable to return. The minimum replacement is not a starting point for negotiation. It is the threshold below which a valid replacement has not been achieved.

The Full Redesign Artifact expands these five components into a seven-part minimum valid pilot by adding pre/post burden measurement and the rule that the pilot must work without NHS integration. The five define the architecture. The seven define the pilot. The NHS recognition pathway is Move 7 — a later migration step, not a minimum replacement condition.

Board decision required

BOARD QUESTION	REQUIRED ANSWER
What is the system failing to do?	Put usable operational memory in front of the clinician before a consequential decision. Not create records — deliver them where the decision is made.
What must stop, and when?	Immediately: leaving the patient as the default memory carrier; measuring success by engagement or adoption; presenting routing output as diagnosis; treating NHS integration as a prerequisite; adding audit features with no retrieval gain; treating the app as the official record.
What must be built?	Continuity record, symptom event layer, consultation packet, outcome append and emergency access — with carer access and non-digital fallback throughout.
What proves success?	Same pressure produces a different response against baseline: history repetition falls, GP reconstruction time falls, outcomes appear in the next packet, carers no longer need a parallel diary.
What invalidates success?	Patients still repeat their history; clinicians do not read the packet; custody moves to an institution; the app presents routing as diagnosis; benefit is shown only for digitally confident early adopters.

SECTION 6

WHAT STOPS IMMEDIATELY

The following are refusal conditions, not optimisation targets. Behaviours that reproduce the failure stop immediately. Structures that reproduce it stop before their replacement becomes the official path. Nothing scales while a refusal condition is still active.

STOP	WHY	WHEN
Treating the patient as the default memory carrier without structural support	The patient performing the memory function is the first structural break. Building on that assumption replicates the break in a new interface.	Immediately
Measuring success by engagement or adoption rather than continuity retrieval at the point of decision	Downloads, sessions and daily use reward documentation volume. The test is whether usable continuity is present when a clinical decision is made. An app designed for daily engagement becomes a health journal.	Immediately — before any metric framework is set

STOP	WHY	WHEN
Presenting routing output as diagnosis or condition detection	The app must never become a diagnostic authority. Detecting conditions, ranking risks or reassuring patients that care is unnecessary moves it into a different, regulated product class.	Immediately — in any interface, marketing or communication
Treating NHS integration as a prerequisite for usefulness	If the app only works with NHS integration, adoption needs institutional permission. The route depends on being useful first.	Immediately — before the roadmap is set
Adding features that improve the audit record without improving retrieval	Features that add documentation without changing whether continuity arrives at the decision repeat the old correction pattern. PDF export is a fallback, not the product.	Immediately — an ongoing design gate
Treating the app as the official NHS record or a replacement for it	Before formal recognition (Move 7), the app is a patient-held continuity layer, not the medico-legal record. Confusing the two creates clinical and regulatory risk.	Immediately — before any public claim of clinical equivalence

These six are the implementation Vanish List in the Full Redesign Artifact, Section 8. The Diagnostic Artifact also names four system-level refusal conditions, including measuring digital programmes by adoption and treating discharge-summary failure as a communication problem.

SECTION 7

DEPENDENCY ORDER

The redesign must happen in the correct sequence. Right fixes in the wrong order recreate the failure at a new stage. Preparatory work may begin early; nothing becomes the official path or scales before its dependency is complete.

MOVE	WHAT HAPPENS	CONSEQUENCE IF SKIPPED
1	Build the truth kernel — the patient onboarding flow and continuity data structure; privacy and security foundations follow directly (Move 1b)	Later layers inherit an undefined continuity object; the kernel cannot be specified retrospectively
2	Build the symptom event layer — structured symptom capture before the encounter; routing prompts follow only once classified	Symptom capture produces isolated data points, with regulatory exposure and no continuity framing
3	Build the consultation packet — one-page summary readable in under 60 seconds	The app becomes a health journal the clinician ignores; the encounter does not change
4	Build the outcome append layer — post-encounter record that travels to the next visit	The loop is not closed; the next encounter begins with reconstruction again
5	Build emergency access — essential fields without login for first responders, plus the Continuity Card	The app fails its most critical clinical moment
6	Build carer access — delegated continuity support with role tagging	Low-digital-literacy patients are excluded; the hidden burden returns in a new form

MOVE	WHAT HAPPENS	CONSEQUENCE IF SKIPPED
7	NHS integration pathway — formal recognition without custody transfer (One-Way Valve)	Attempted before the migration signal, it becomes a procurement process without evidence of value

Moves 5 and 6 both build on Moves 1–4 and can proceed side by side. **NHS integration is Move 7, not Move 1.** The redesign must demonstrate value independently of NHS cooperation before it migrates toward institutional recognition. Move 7 is deferred until patients are arriving with more usable information than the institutional record provides.

GP adoption hypothesis. The redesign assumes GPs will read the packet because it makes the short appointment more usable, not because they are required to. The packet does not instruct the clinician; it prepares them. Whether clinicians actually use it is a pilot measure with a defined failure threshold, not an assumption.

SECTION 8

FAKE PROGRESS TEST

The redesign can be implemented in appearance while the old failure continues in structure. These conditions show the primitive has returned, not that the redesign has succeeded:

CONDITION	WHAT IT MEANS
The app exists but the GP still reconstructs history at the start of the consultation	The app is a health journal, not a continuity system.
Clinicians do not read or act on the packet	The packet is a document, not a mechanism. The consultation has not changed.
Success is measured by downloads and engagement rather than continuity retrieval	Growth metrics reward the audit layer. The test is retrieval before the clinical decision.
Patients still repeat their full history as the default	Burden has not moved from people into structure. The proof condition has not been met.
NHS integration requires custody transfer to the institution	The custody invariant is broken. The broken architecture has been rebuilt in a new wrapper.
Routing output is presented as a diagnosis on any surface or in any communication	The app has entered clinical authority it cannot hold, with regulatory classification risk.
The redesign is declared complete before baselines were taken	Without a baseline, same pressure → different response cannot be shown.

SECTION 9

CONSEQUENCE OF DEFERRAL

Deferral does not preserve a neutral status quo. It preserves the hidden burden: patients, carers, known GPs and reception staff continue carrying continuity functions the architecture has not formalised. When any carrier withdraws — the patient cannot recall their history, the carer is unavailable, the known GP has left — the structural gap can surface as a clinical event.

The consequence environment visible in public evidence is likely to persist: diagnostic delay, coroners' concern about information sharing, patient-carried memory burden and GP workforce pressure. Digital programmes that address a downstream symptom without correcting the primitive remain liable to stall or be replaced, leaving the same failure in new packaging.

COST CLASS	WHAT THE DIAGNOSTIC SHOWS (CONSEQUENCE ENVIRONMENT, NOT CAUSED COST)
Clinical negligence	£2.8 billion paid across all NHS Resolution schemes in 2023/24 — an NHS-wide figure. In the GP scheme's first year (2019/20, 401 cases), delay or failure to diagnose featured in 43.5% of cases, with NHS Resolution cautioning against firm conclusions from one year.
Preventable harm	The Guardian reported 36 coroners' warnings in 2024 on NHS information sharing. Coroners continue to raise record-sharing with national bodies.
GP workforce	76% of GPs say excessive workload compromises patient safety; 42% say they are unlikely to be in general practice in five years (RCGP, 2024). Workforce exit can intensify the loss of the relational continuity that was masking the failure.
Digital programme outcomes	The National Programme for IT had a forecast total cost of £9.8 billion and did not deliver its central aim of a national electronic patient record (Public Accounts Committee, 2013).

SECTION 10

PROOF CONDITIONS

Proof is not that the app exists. Proof is not a go-live date. Proof is behavioural.

> **PROOF STANDARD**
>
> **Same pressure → different response.** The redesign is real only when, under the same workload and patient complexity, the patient no longer repeats their history, the GP begins at clinical judgment rather than reconstruction, and the outcome is preserved for the next encounter.

Before a pilot begins, baselines are captured: patient history repetition, GP time on verbal reconstruction, discharge information present before follow-up, and carer diary maintenance. These are target measurement requirements; without a baseline, same pressure → different response cannot be shown. The pilot compares against matched practices or a staggered rollout, with measures and analysis set in advance, so that change can be distinguished from chance and from unrelated workflow changes.

The redesign is confirmed when all of the following hold:

- → The patient arrives with the packet; the GP reads it in under 60 seconds; the appointment begins at clinical judgment
- → Patients no longer routinely repeat their full history at each encounter
- → Continuity holds when the patient cannot tell the story — unconscious, acutely distressed or cognitively impaired — through emergency access, carer access or the printed Continuity Card
- → Carers no longer keep separate health diaries as the main continuity mechanism; the hidden burden falls when its labour becomes avoidable
- → The outcome is appended after each encounter and appears in the next packet without the patient reconstructing it
- → Digitally excluded, elderly and cognitively impaired patients receive the same continuity benefit through carer-assisted access, printed packets, PDF export or the Continuity Card

→ High-burden patients — repeat presenters, post-discharge patients, people with several long-term conditions — show measurable improvement against baseline, not only digitally confident early adopters

SECTION 11

BOARD VERDICT

BOARD VERDICT

Replace patient-carried continuity with a patient-held operational continuity layer. The patient holds custody. The system carries memory and preparation. The clinician carries authority. The consultation packet arrives before the encounter; the outcome is appended after it. The official NHS GP record remains the medico-legal record; institutions may later recognise or ingest the continuity record, but they do not own the continuity primitive. The redesign is real only when patients no longer repeat their history as the default — and when continuity holds even when the patient cannot tell the story.

The board choice is whether to correct the primitive or keep correcting inside it. The programmes reviewed in the Diagnostic improved documentation, access or sharing without establishing that usable continuity reaches the decision. Those improvements remain useful; they are not a substitute for primitive correction.

Next steps for the board

1. Formally resolve and lock the governing diagnosis before implementation-level redesign begins.
2. Where a Full Structural Engagement is commissioned, use Artifact 04 as the restricted builder specification for the seven-move transition sequence, invariants, drift gates, admissibility conditions and builder handoff.
3. Treat the Diagnostic Resolution Record and the Full Redesign acceptance record as exemplar governance devices, not universal contract forms.

WHAT THIS DEMONSTRATES

This executive summary demonstrates the replacement logic of a structural redesign. Artifact 04 sits underneath it with the restricted builder specification, transition logic, invariants and drift gates. This public summary is not sufficient for implementation.

Delivery model. This redesign is specified independently of delivery model. The continuity record must remain open and non-proprietary. Patient control of the record and vendor-neutral continuity are non-negotiable, whoever builds or operates the system.

Source note. Figures in Section 9 are drawn from the Diagnostic Artifact, which sources them to NHS Resolution (Annual Report 2023/24; CNSGP year-one report, 2022), The Guardian (November 2024), RCGP (2024) and the Public Accounts Committee (2013). This executive summary introduces no independent quantitative claims. To discuss access to the restricted Full Redesign Artifact, contact **hello@jamieforrester.com** · jamieforrester.com

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